

GLOBAL:
Blue box: Represents live text

GLOBAL:
Orange indicates "selected state"
for all tabs

BUTTON TAB:
Opens 13.1 - MTX-IR - AIM

BUTTON TAB:
Opens 13.2 - MTX-IR - ACQUIRE

BUTTON TAB:
Opens 13.3 - TNFi-IR - ATTAIN

Opens 8.2.1 - AIM Study Design

Opens 13.2.1 - ACQUIRE
Study Design

Opens 13.3.1 - ATTAIN
Study Design

OVERVIEW

AIM

ACQUIRE

ATTAIN

IN MODERATE TO SEVERE RA

ORENCIA® (abatacept) has been studied across multiple clinical trials

AIM MTX-IR²⁰

A 12-month, phase III, multinational, double-blind, placebo-controlled randomized study

652
ADULT
MTX-IR
PATIENTS

ORENCIA IV + MTX (n=433)

Placebo + MTX (n=219)

AIM STUDY DESIGN

ACQUIRE IV/SC²³

A 6-month, phase IIIb, multinational, randomized, double-blind, double-dummy, noninferiority study

1457
ADULT
MTX-IR
PATIENTS

ORENCIA IV + MTX (n=721)

ORENCIA SC + MTX (n=736)

ACQUIRE STUDY DESIGN

ATTAIN TNFi-IR²⁴

A 6-month, phase III, randomized, multicenter, double-blind, placebo-controlled study

389
ADULT
TNFi-IR
PATIENTS

ORENCIA IV + DMARDs (n=258)

Placebo + DMARDs (n=133)

ATTAIN STUDY DESIGN

Selected Important Safety Information

Pregnant and Nursing Mothers:

ORENCIA® (abatacept) should be used during pregnancy only if clearly needed. The risk for development of autoimmune diseases in humans exposed in *utero* to abatacept has not been determined. Nursing mothers should be informed of the risk/benefit of continued breast-feeding or discontinuation of the drug. A pregnancy registry has been established to monitor fetal outcomes. Healthcare professionals are encouraged to register pregnant patients exposed to ORENCIA by calling 1-877-311-8972.

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IMPORTANT SAFETY INFORMATION

FULL PRESCRIBING INFORMATION

Opens 13.0 - Additional Efficacy Data

Opens 13.2 - MTX-IR - ACQUIRE

Opens 13.3 - TNFi-IR - ATTAIN

Opens 8.2.1 - AIM Study Design

OVERVIEW

AIM

ACQUIRE

ATTAIN

CO-PRIMARY ENDPOINTS^{12,20}

Erosion score at 1 year ($P<0.01$)

0.61 ORENCIA IV + MTX (n=391)
1.47 Placebo + MTX (n=195)

ACR 20 at 6 months ($P<0.001$)

68% ORENCIA IV + MTX (n=424)
40% Placebo + MTX (n=214)

HAQ-DI at 1 year ($P<0.001$)

Proportion of patients with ≥ 0.3 improvement
64% ORENCIA IV + MTX (n=424)
39% Placebo + MTX (n=214)
HAQ-DI, Health Assessment Questionnaire-Disability Index.

AIM STUDY DESIGN

OTHER ENDPOINTS^{12,20}

ORENCIA IV + MTX demonstrated significant ACR 20/50/70 responses at 1 year vs MTX alone¹²

ORENCIA IV + MTX (n=424)

Placebo + MTX (n=214)

$P<0.001$.

73%

40%

48%

18%

29%

6%

ACR 20

ACR 50

ACR 70

Opens 13.1.2 - Enlarge Chart Overlay (ACR)

Selected Important Safety Information

MORE +

Most Serious Adverse Reactions:

Serious infections (3% ORENCIA® (abatacept) vs 1.9% placebo) and malignancies (1.3% ORENCIA vs 1.1% placebo).

Malignancies:

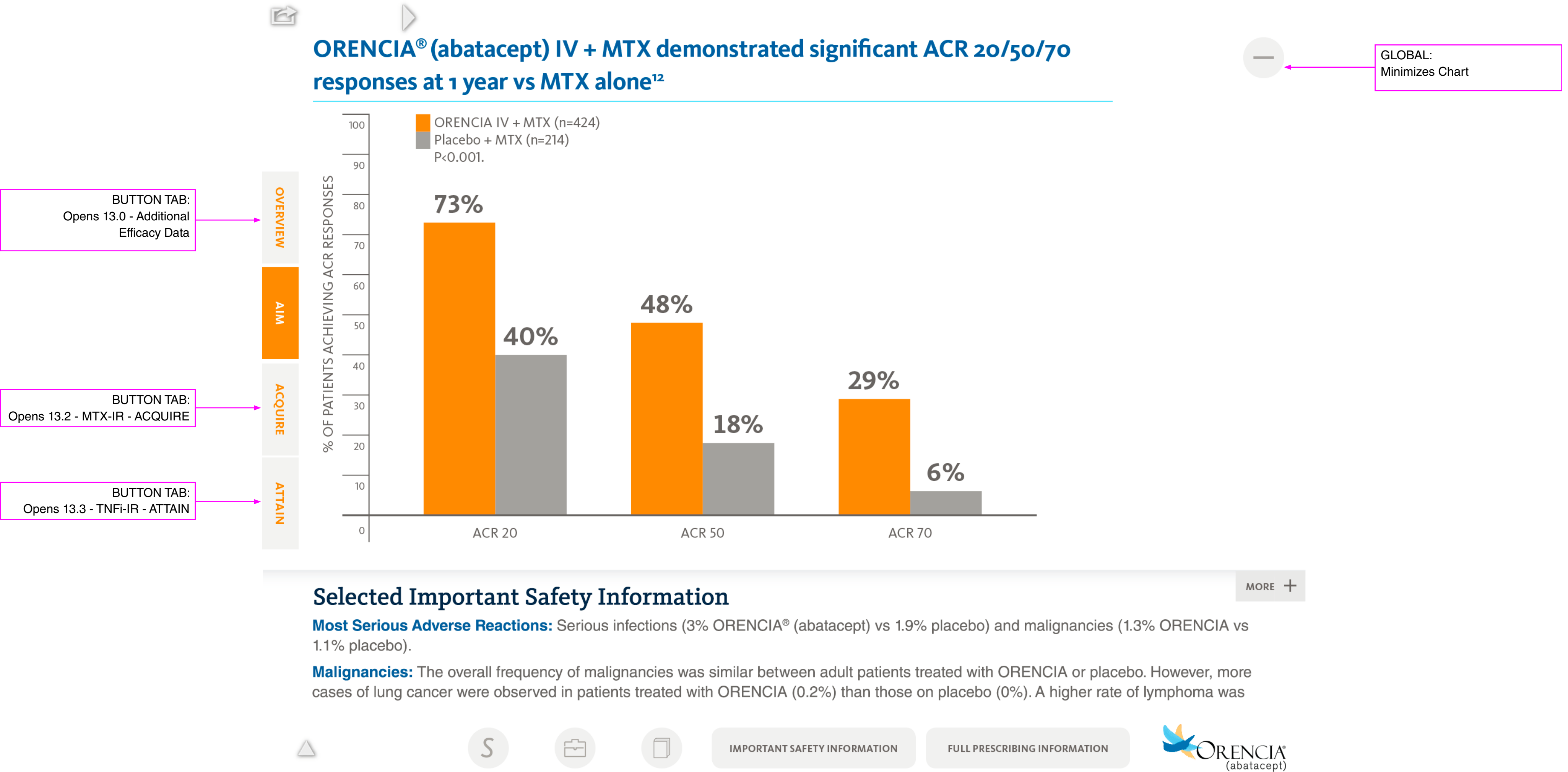
The overall frequency of malignancies was similar between adult patients treated with ORENCIA or placebo. However, more cases of lung cancer were observed in patients treated with ORENCIA (0.2%) than those on placebo (0%). A higher rate of lymphoma was

IMPORTANT SAFETY INFORMATION

FULL PRESCRIBING INFORMATION

ORENCIA®

(abatacept)





Selected Important Safety Information

COLLAPSE —

Most Serious Adverse Reactions: Serious infections (3% ORENCIA® (abatacept) vs 1.9% placebo) and malignancies (1.3% ORENCIA vs 1.1% placebo).

Malignancies: The overall frequency of malignancies was similar between adult patients treated with ORENCIA or placebo. However, more cases of lung cancer were observed in patients treated with ORENCIA (0.2%) than those on placebo (0%). A higher rate of lymphoma was seen compared to the general population; however, patients with RA, particularly those with highly active disease, are at a higher risk for the development of lymphoma. The potential role of ORENCIA in the development of malignancies in humans is unknown.



IMPORTANT SAFETY INFORMATION

FULL PRESCRIBING INFORMATION



IN MODERATE TO SEVERE RA

Powerful symptom relief as an intravenous (IV) or subcutaneous (SC) formulation

BUTTON TAB:
Opens 13.0 - Additional
Efficacy Data

OVERVIEW

BUTTON TAB:
Opens 13.1 - MTX-IR - AIM

AIM

BUTTON TAB:
Opens 13.3 - TNFi-IR - ATTAIN

ACQUIRE

Opens 13.2.1 - ACQUIRE
Study Design

ATTAIN

PRIMARY ENDPOINT^{12,23}
Noninferiority based on ACR 2o at 6 months

76% ORENCIA SC + MTX (n=693)

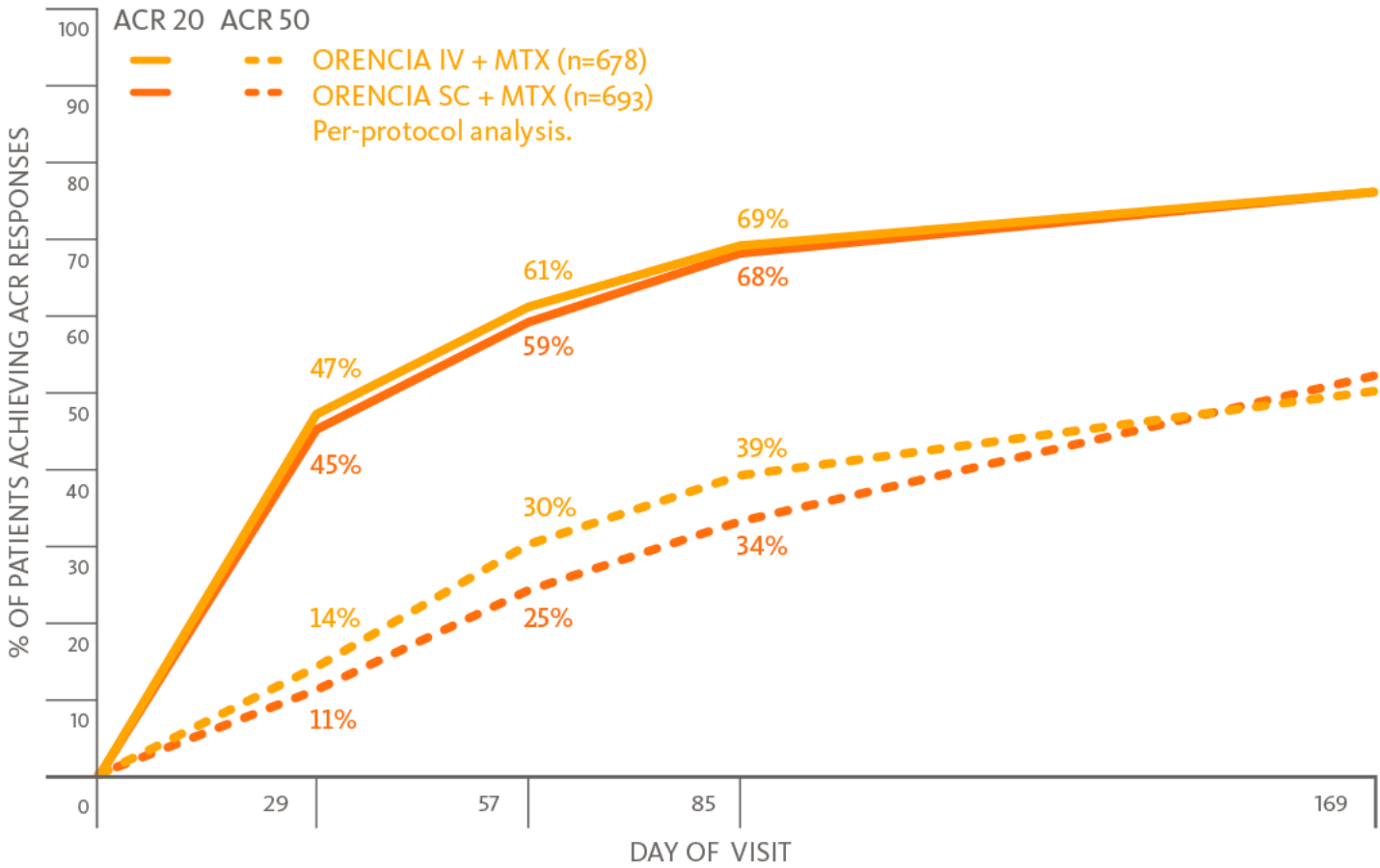
76% ORENCIA IV + MTX (n=678)

95% CI: -4.2, 4.8 based on pre-specified margin for noninferiority of -7.5% for the SC group.

The same patients may not have responded at each time point.

ACQUIRE STUDY DESIGN

PRIMARY AND SECONDARY ENDPOINTS^{12,23,25}
ORENCIA IV and ORENCIA SC ACR responses over 6 months



Day of Visit	ORENCIA IV + MTX (n=678) ACR 20	ORENCIA IV + MTX (n=678) ACR 50	ORENCIA SC + MTX (n=693) ACR 20	ORENCIA SC + MTX (n=693) ACR 50
0	0%	0%	0%	0%
29	47%	14%	45%	11%
57	61%	30%	59%	25%
85	69%	39%	68%	34%
169	76%	54%	76%	54%

Opens 13.2.2 - Enlarge
Chart Overlay

Selected Important Safety Information

Concomitant Use with TNF Antagonists: Concurrent therapy with ORENCIA® (abatacept) and a TNF antagonist is not recommended. In controlled clinical trials, adult patients receiving concomitant intravenous ORENCIA and TNF antagonist therapy experienced more infections (63%) and serious infections (4.4%) compared to patients treated with only TNF antagonists (43% and 0.8%, respectively), without an important enhancement of efficacy.

IN MODERATE TO SEVERE RA

GLOBAL:
Minimizes Study Design

ACQUIRE²³

6-month, phase IIIb, multinational, randomized, double-blind, double-dummy, noninferiority study

PATIENTS

1457 adult patients with moderate to severe RA who had an inadequate response to MTX

INCLUSION CRITERIA

- Met the ACR criteria for RA
- Inadequate response to ≥3 months of MTX therapy (≥15 mg/week)
- ≥10 swollen joints, ≥12 tender joints, and C-reactive protein (CRP) levels of ≥0.8 mg/dL at randomization

TREATMENT ARMS

ORENCIA IV + MTX (n=721)
ORENCIA SC + MTX (n=736)

Patients were randomly assigned in a 1:1 ratio to receive ORENCIA IV (~10 mg/kg based on patient weight) + MTX or ORENCIA SC + MTX. ORENCIA IV (10 mg/kg) was administered on days 1, 15, and 29 and every 4 weeks thereafter. ORENCIA SC (125 mg/week) was administered weekly with an IV loading dose on day 1.

PRIMARY ENDPOINT



Noninferiority of ORENCIA SC to ORENCIA IV based on ACR 20 response at 6 months (per-protocol population)

SECONDARY ENDPOINTS

- ACR 50 and ACR 70 responses at 6 months
- Proportion of patients achieving a ≥0.3 improvement from baseline in HAQ-DI at 6 months

Selected Important Safety Information

Most Frequent Adverse Events (≥10%): Headache, upper respiratory tract infection, nasopharyngitis, and nausea were the most commonly reported adverse events in the adult RA clinical studies.

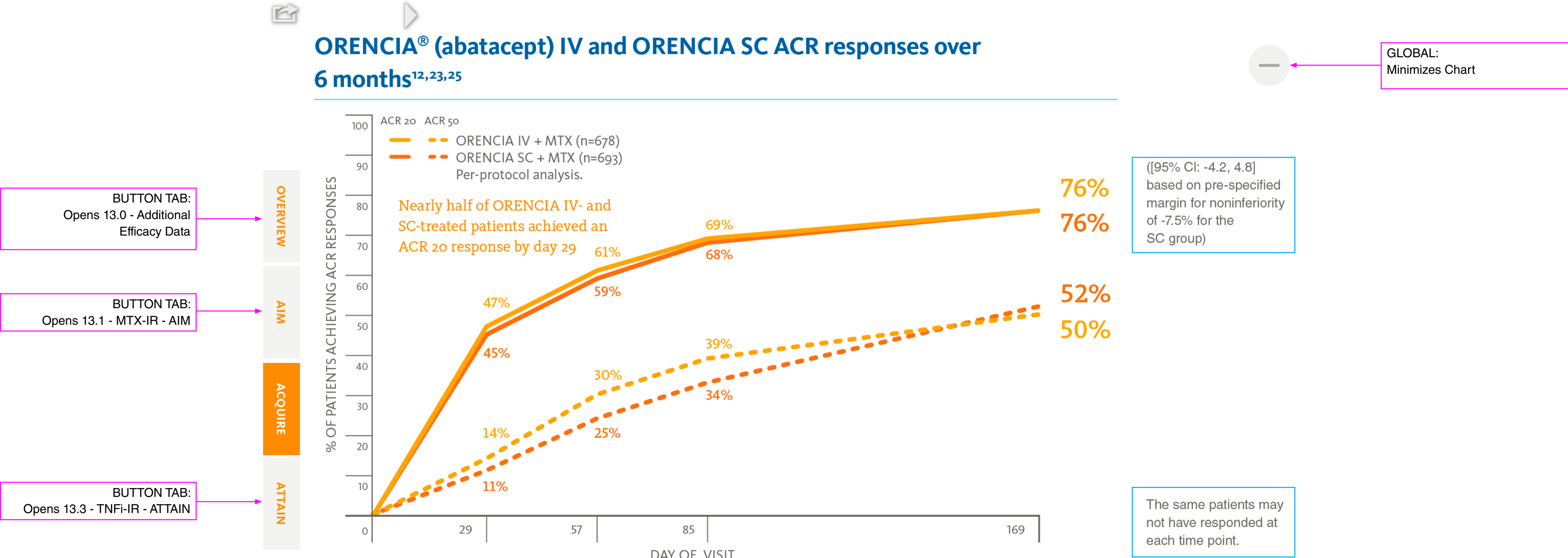
Note concerning SC ORENCIA: The safety and efficacy of SC ORENCIA® (abatacept) have not been studied in patients under 18 years of age.



IMPORTANT SAFETY INFORMATION

FULL PRESCRIBING INFORMATION





Selected Important Safety Information

Most Frequent Adverse Events (≥10%): Headache, upper respiratory tract infection, nasopharyngitis, and nausea were the most commonly reported adverse events in the adult RA clinical studies.

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Opens 13.3.1 - ATTAIN Study Design

OVERVIEW

AIM

ACQUIRE

ATTAIN

CO-PRIMARY ENDPOINTS^{12,24}

ACR 20 at 6 months ($P<0.001$)

50% ORENCIA IV + DMARDs (n=256)
20% Placebo + DMARDs (n=133)

HAQ-DI at 6 months ($P<0.001$)

Proportion of patients with ≥ 0.3 improvement

47% ORENCIA IV + DMARDs (n=256)
23% Placebo + DMARDs (n=133)

The same patients may not have responded at each time point.

ATTAIN STUDY DESIGN

PRIMARY AND SECONDARY ENDPOINTS^{12,24}

ORENCIA IV + DMARDs significantly improved ACR 20/50/70 responses at 6 months vs DMARDs alone

Endpoint	ORENCIA IV + DMARDs (n=256)	Placebo + DMARDs (n=133)
ACR 20 (PRIMARY ENDPOINT)	50%*	20%
ACR 50 (SECONDARY ENDPOINT)	20%*	4%
ACR 70 (SECONDARY ENDPOINT)	10%†	2%

* $P<0.001$. † $P<0.01$.

Opens 13.3.2 - Enlarge Chart Overlay

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Selected Important Safety Information

Concomitant Use with TNF Antagonists:

Concurrent therapy with ORENCIA® (abatacept) and a TNF antagonist is not recommended. In controlled clinical trials, adult patients receiving concomitant intravenous ORENCIA and TNF antagonist therapy experienced more infections (63%) and serious infections (4.4%) compared to patients treated with only TNF antagonists (43% and 0.8%, respectively), without an important enhancement of efficacy.

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IMPORTANT SAFETY INFORMATION

FULL PRESCRIBING INFORMATION

IN MODERATE TO SEVERE RA

ATTAIN²⁴

6-month, phase III, multicenter, randomized, double-blind, placebo-controlled study

PATIENTS

389 adult patients with active moderate to severe RA and an inadequate response to at least 3 months of anti-TNF therapy

INCLUSION CRITERIA

- ≥18 years of age with active RA for ≥1 year
- Inadequate response to anti-TNF-α therapy after at least 3 months of treatment
- ≥12 tender and ≥10 swollen joints, C-reactive protein (CRP) ≥1 mg/dL at randomization
- Prior exposure to oral DMARDs or anakinra for ≥3 months, and stable dose for ≥28 days

TREATMENT ARMS

ORENCIA® (abatacept) IV + DMARDs (n=258)
Placebo + DMARDs (n=133)

Patients were randomly assigned in a 2:1 ratio to receive ORENCIA IV (~10 mg/kg based on patient weight) + DMARDs (n=258) or placebo + DMARDs (n=133). ORENCIA or placebo was administered on days 1, 15, and 29, and every 4 weeks thereafter, up to and including day 141. Patients discontinued anti-TNF-α therapy before randomization.

PRIMARY ENDPOINT

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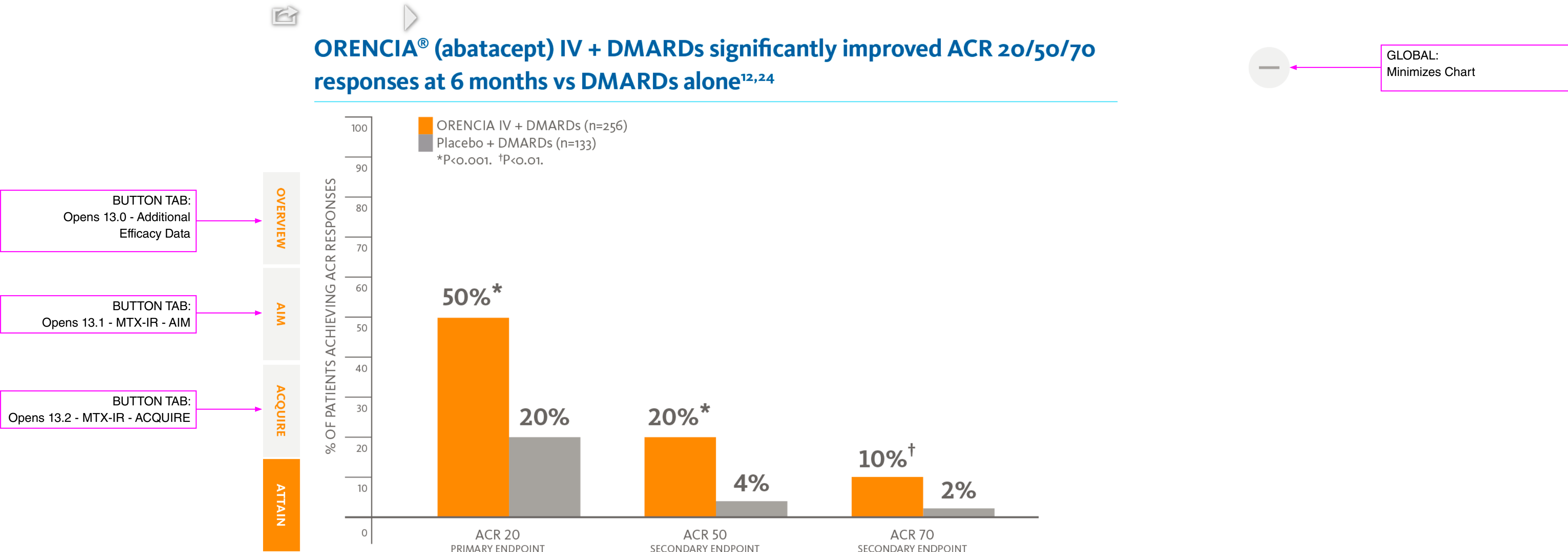
Proportion of patients achieving an ACR 20 response at 6 months
- 

Proportion of patients achieving a ≥0.3 improvement from baseline in HAQ-DI at 6 months

GLOBAL:
Minimizes Study Design

Selected Important Safety Information

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